

August 20, 2026

Aetna
Provider Resolution Team
P.O. Box 14020
Lexington, KY 40512

RE: Formal Appeal of Claim Denial — Marc Corkery (MRN MUSC8042994), Claim MUSC-76E3-2

Patient	Marc Corkery (DOB 1977-08-10)
MRN	MUSC8042994
Member ID / Group	AET942376912 / GRP-91373
Claim number	MUSC-76E3-2
Date of service	2026-02-21
Procedure billed	CPT 99284 — Emergency department visit, moderate-to-high severity
Primary diagnosis	R69 — Illness, unspecified
Denied amount	\$2,656.13
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-09-29

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this Level 1 reconsideration appeal on behalf of patient Marc Corkery (MRN: MUSC8042994, Member ID: AET942376912, Group: GRP-91373) regarding claim MUSC-76E3-2 for services rendered on February 21, 2026, at MUSC Health Rutledge Tower Ambulatory Care. Aetna denied the claim in full on April 2, 2026, citing CARC 50 and RARC N115, with the payer remark stating: "Documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and requests immediate reversal and payment at the allowed amount of \$1,200.32.

CLINICAL BACKGROUND

Mr. Marc Corkery, a 49-year-old male with an active diagnosis of obesity (E66.9, BMI documented at 30.27 kg/m²), presented to the MUSC Health emergency department on February 21, 2026, and was evaluated by Dr. Priya N. Raghavan, MD (NPI: 1669504132). The encounter was coded at CPT 99284, reflecting an emergency department visit of moderate-to-high severity, billed under revenue code 0450 at place of service 23. Mr. Corkery's active medication regimen includes a fluticasone propionate metered dose inhaler and an albuterol metered dose inhaler, indicating an ongoing respiratory condition requiring chronic pharmacologic management. This background is clinically relevant to the acuity of any acute presentation and supports the moderate-to-high severity level assigned to the visit.

BASIS FOR APPEAL

Aetna's denial rests on the assertion that the documentation does not establish medical necessity for the service as reported. MUSC Health contends that this determination is inconsistent with the clinical record and the applicable standard of care for emergency department evaluation.

First, CPT 99284 is appropriately supported by the complexity of the encounter. The patient's active comorbidities — including obesity and a chronic respiratory condition requiring dual bronchodilator and inhaled corticosteroid therapy — represent a meaningful baseline burden that directly informs the medical decision-making complexity required of the treating physician. Emergency department level-of-service coding is governed by the nature of the presenting problem and the medical decision-making or time documented, not solely by the primary diagnosis code assigned at discharge.

Second, the use of ICD-10 code R69 (Illness, unspecified) as the primary diagnosis reflects the clinical reality that a definitive diagnosis is not always established at the time of an emergency department visit. Coding guidelines explicitly recognize R69 as a valid code for encounters in which the nature of the illness has not been determined. The absence of a more specific final diagnosis does not, in itself, negate medical necessity; it reflects appropriate and honest coding of an acute presentation under evaluation.

Third, RARC N115 indicates the denial was based on a Local Coverage Determination. MUSC Health requests that Aetna identify the specific LCD applied and confirm that it is applicable to this commercial plan, this place of service, and this patient's clinical circumstances, as LCD applicability to commercial emergency department claims is not automatic.

MUSC Health is prepared to submit the complete encounter documentation, including the treating physician's notes, triage records, and any diagnostic workup performed, upon request or as directed through the Availity Disputes and Appeals portal.

REQUESTED ACTION

MUSC Health respectfully requests that Aetna conduct a full clinical review of the submitted documentation, reverse the medical necessity denial for claim MUSC-76E3-2, and issue payment at the contracted allowed amount of \$1,200.32. Should Aetna require additional clinical records or a peer-to-peer review with Dr. Raghavan, we ask that the appropriate contact be provided promptly so that this matter may be resolved prior to the appeal deadline of September 29, 2026.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record